

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Levonorgestrel 1.5mg tablets (Levonelle) and Ulipristal acetate 30mg tablets (ellaOne) for the treatment of Emergency Contraception

Patient Group Direction, version 003, issued 11 September 2026. Supersedes Version 002, 10 September 2026.

Summary of NICE / NICE CKS Guidelines for Emergency Contraception

Overview

Emergency contraception is used to prevent pregnancy after unprotected sexual intercourse or contraceptive failure.

Assessment

The following assessment factors should be considered:

- Time since unprotected sexual intercourse (UPSI)
- Previous UPSI in the cycle
- Current contraception use
- Date of last menstrual period
- Regular cycle length
- Potential drug interactions (enzyme inducers)
- BMI and weight
- Breastfeeding status

Management

- **Levonorgestrel:** Within 72 hours (3 days) of UPSI; most effective within 12 hours
- **Ulipristal acetate:** Within 120 hours (5 days) of UPSI
- Consider copper IUD as most effective emergency contraception option (refer to GP/clinic)

Red flags

- Suspected existing pregnancy
- Severe asthma (ulipristal)
- Repeated emergency contraception requests (signpost to ongoing contraception)

Patient Group Direction

for the supply/administration of
Levonorgestrel 1.5mg tablet (Levonelle)
for the treatment of
Emergency Contraception

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Emergency contraception within 72 hours of unprotected sexual intercourse
Inclusion criteria	Females of reproductive age. Under 13: any sexual activity is a safeguarding concern; supply may still be appropriate but a safeguarding referral is MANDATORY. Aged 13 to 15: assess and record Fraser competence, ask about coercion, the age of the partner and any safeguarding concern, and follow the local safeguarding pathway. Record the assessment. Within 72 hours of unprotected sexual intercourse (UPSI) or contraceptive failure Valid informed consent obtained No known or suspected pregnancy
Exclusion criteria	Known or suspected pregnancy

	Hypersensitivity to levonorgestrel or any component of the formulation Severe hepatic impairment (e.g., cirrhosis) Hereditary galactose intolerance Acute intermittent porphyria
Cautions	Enzyme-inducing drugs in the last 4 weeks (anticonvulsants, rifampicin, antiretrovirals, St John's Wort): offer a copper IUD; if declined give levonorgestrel 3 mg (two tablets, licensed). Do NOT switch to ulipristal, which is not recommended in these women. Malabsorption conditions affecting drug absorption (e.g., Crohn's disease) History of ectopic pregnancy Breastfeeding - avoid for 8 hours after dose Weight 70 kg or over, or BMI 26 or over: ulipristal is preferred (FSRH) unless unsuitable; where levonorgestrel is used give 3 mg (double dose), which is off-label per FSRH guidance. Explain this and record it. Recent unprotected intercourse where pregnancy risk is higher (e.g., mid-cycle)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Levonorgestrel 1.5mg tablet (Levonelle)
Legal category	P (Pharmacy medicine) for women aged 16 and over; POM under 16. This PGD is the legal authority for supply to those under 16; for those 16 and over it sets the standard of assessment and record for what is otherwise a P sale.
Storage	Store below 25°C in a dry place. Keep in original packaging.
Route/method of administration	Oral - swallow tablet whole with water
Dose and frequency	Standard dose: 1.5mg as a single dose, taken as soon as possible after UPSI (ideally within 12 hours, but effective up to 72 hours) Double dose: 3 mg where taking enzyme-inducing drugs (licensed), or where weight is 70 kg or over or BMI 26 or over (off-label, FSRH). Record the reason.
Quantity to be administered and/or supplied	1 tablet (1.5mg single treatment) or 2 tablets (3mg if double dose indicated)
Maximum or minimum	Single dose only; repeated use not recommended within the same

treatment period	cycle
Adverse effects	Nausea and vomiting (if vomiting occurs within 3 hours of taking the tablet, another tablet must be taken) Abdominal pain and abdominal discomfort Headache Fatigue and tiredness Dizziness and vertigo Breast tenderness Menstrual irregularity (period may be delayed or early) Mood changes (rare) Pelvic pain

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	If vomiting occurs within 3 hours of taking the tablet, return immediately as another tablet is needed. Your next period may be earlier or later than expected. If your period is more than 7 days late, perform a pregnancy test or contact your

	doctor. Use barrier contraception (condoms) until your next period to reduce risk of pregnancy. Consider starting or continuing regular contraception (after waiting 5 days if you have taken ulipristal) - discuss options with your doctor or healthcare provider. This emergency contraception does not protect against sexually transmitted infections (STIs). Discuss STI testing if appropriate. If you experience any adverse effects or have concerns, contact your doctor or healthcare provider. Remember that emergency contraception works best when taken as soon as possible after unprotected sexual intercourse.
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Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017
<https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017
<https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Emergency Contraception
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date		
11/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction
for the supply/administration of
Ulipristal acetate 30mg tablet (ellaOne)
for the treatment of
Emergency Contraception

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Emergency contraception within 120 hours of unprotected sexual intercourse
Inclusion criteria	Females of reproductive age. Under 13: any sexual activity is a safeguarding concern; supply may still be appropriate but a safeguarding referral is MANDATORY. Aged 13 to 15: assess and record Fraser competence, ask about coercion, the age of the partner and any safeguarding concern, and follow the local safeguarding pathway. Record the assessment. Within 120 hours (5 days) of unprotected sexual intercourse (UPSI) or contraceptive failure Valid informed consent obtained No known or suspected pregnancy

Exclusion criteria	Known or suspected pregnancy Hypersensitivity to ulipristal acetate or any component of the formulation Severe hepatic impairment (e.g., cirrhosis) Severe asthma insufficiently controlled by oral glucocorticoids Hereditary galactose intolerance Enzyme-inducing drugs in the last 4 weeks (anticonvulsants, rifampicin, antiretrovirals, St John's Wort): ulipristal is not recommended (SmPC). Offer a copper IUD or levonorgestrel 3 mg.
Cautions	Progestogen-containing contraceptive taken in the previous 7 days: may reduce ulipristal efficacy; consider levonorgestrel instead. Hormonal contraception must not be restarted until 5 days after ulipristal, with condoms until it is reliable again. Malabsorption conditions affecting drug absorption History of ectopic pregnancy Breastfeeding - avoid breastfeeding for 7 days after dose Repeated use in the same cycle not recommended Hormonal contraception should not be started until 5 days after taking ulipristal
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Ulipristal acetate 30mg tablet (ellaOne)
Legal category	P (Pharmacy medicine). ellaOne is licensed for sale without prescription at any age of reproductive potential; this PGD sets the standard of assessment and record for the supply.
Storage	Store below 25°C. Keep in original packaging.
Route/method of administration	Oral - swallow tablet whole with water
Dose and frequency	30mg as a single dose, taken as soon as possible after UPSI (effective up to 120 hours/5 days)
Quantity to be administered and/or supplied	1 tablet (30mg single treatment)
Maximum or minimum treatment period	Single dose only; repeated use not recommended within the same cycle

Adverse effects	Headache Nausea Abdominal pain and abdominal discomfort Dysmenorrhoea (painful period) Fatigue and tiredness Dizziness and vertigo Mood changes and mood swings Myalgia (muscle pain) Back pain Breast tenderness Menstrual irregularity Pelvic pain
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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	Your next period may be earlier or later than expected. If your period is more than 7 days late, perform a pregnancy test or contact your doctor. Use barrier contraception (condoms) until your next period to reduce risk of pregnancy. Consider starting or continuing regular contraception (after waiting 5 days if you have taken ulipristal) - discuss options with your doctor or healthcare provider. This emergency contraception does not protect against sexually transmitted infections (STIs). Discuss STI testing if appropriate. If you experience any adverse effects or have concerns, contact your doctor or healthcare provider. Remember that emergency contraception works best when taken as soon as possible after unprotected sexual intercourse.
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Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Legal categories corrected: levonorgestrel 1.5 mg is P at 16 and over and POM under 16; ulipristal acetate 30 mg (ellaOne) is P. Version 002 called both POM.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. The ulipristal arm previously carried two expiry dates (31/7/27 and 31/10/26).

Safeguarding: the inclusion criterion admitted girls from age 11 with no safeguarding provision. Under 13 now carries a mandatory safeguarding referral; 13 to 15 requires a recorded Fraser competence assessment and safeguarding questions.

Enzyme inducers: the levonorgestrel caution told the pharmacist to consider switching to ulipristal, which the ellaOne SmPC says is not recommended after enzyme inducer use in the previous 4 weeks. Both arms now direct to a copper IUD or levonorgestrel 3 mg (licensed). Enzyme inducer use is an exclusion for ulipristal.

Ulipristal: 'concurrent or recent use of hormonal contraceptives' was an exclusion while 'contraceptive failure' was an inclusion, so every woman presenting after a pill failure was both included and excluded. Replaced with the FSRH position as a caution.

Vomiting: both SmPCs require another tablet if vomiting occurs within 3 hours; the document said 2 hours in three places. Corrected.

Weight-based 3 mg levonorgestrel is now labelled as off-label per FSRH, with ulipristal preferred at 70 kg or over or BMI 26 or over.

Signed on behalf of Get Real Health

Version v003, 11 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		11 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		11 September 2026

Both authorising signatories reviewed this version together on 11 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.